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Biopharma | North America

Front-line lung cancer takeaways from ASCO

ASCO conference featured a number of presentations of Ph2/3 data emerging in front-line NSCLC, which is currently dominated by MRK's Keytruda+/-chemo. We were most focused on MRK/Kelun's Sac-TMT (TROP2 ADC) and Akeso/SMMT's Ivo (PD1xVEGF bispec). See below and within for our takeaways.

At ASCO (see preview [HERE](#)) we were focused on MRK/Kelun's sacituzumab tirumotecan (sac-TMT; ADC to TROP2), and the PD(L)1xVEGF bispecifics - Akeso/SMMT's ivonescimab, BMY/BNTX's pumitamic, and PFE's PF'4404. These two modalities have emerged as some of the leading strategies to attempt to improve upon or replace Keytruda (PD1 antibody, which goes off patent in 2028). MRK is uniquely positioned in that it has a drug in each category (including LM-299, a PD-1xVEGF bispecific antibody from LaNova).

MRK (EW): We view the Sac-TMT OpiTROP-Lung05 Ph3 data (PD-L1 positive patients) as an incremental positive update for the drug, but generally consistent with the data released in the abstract (see [HERE](#)), and we see Ph3 data later this year from the OpiTROP-Lung06 China trial (vs. Keytruda+chemo in NSQ PD-L1 negative population) as another key input into the drug's profile. We also await further details in terms of MRK's strategy to address PD-L1 low population. For MRK we model sac-TMT 2035 risk-adjusted sales of \$4.1bn (at a 55% POS) across tumor types vs. VA cons' of \$5.4bn. Additional lateral data on TROP2's in 1L NSCLC will come from AstraZeneca and GILD (see [Exhibit 1](#) below).

PD(L)1xVEGF bispecifics: The data sets at ASCO from Akeso/SMMT, BNTX/BMY, and PFE demonstrate that PD(L)1xVEGF bispecifics can elicit meaningful response across all PD-L1 levels, including TPS<1%. **In addition, we note there were two firsts from this drug class at the conference:** (1) first demonstration of a stat sig survival benefit (SMMT/Akeso's HARMONI-6), and (2) the first global data in 1L NSCLC (BMY/BNTX's ROSETTA-Lung02 Ph2 trial). However, a number of questions are still outstanding in our view and we await more definitive survival data from global trials vs. Keytruda+chemo. We continue to see BNTX (OW) as the best way to gain exposure to this class in our coverage.

Importantly, the Ivo+chemo HARMONI-6 (squamous all comers) OS data represents the first validation of translatability of PFS to OS for the class, but there are a number of caveats noted by the discussant and in the Lancet publication. The discussant stated that while the results were provocative, there are several outstanding questions, including the shorter follow-up, benefit by age, and whether these results can be reproduced in a global study (i.e., HARMONI-3) vs. Keytruda+chemo. **Hence, in our opinion OS data from the ongoing Ivo+chemo HARMONI-3 (all comers) Ph3 trial in 2H26 are more important for the outlook of PD(L)-**

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1xVEGF class given the global nature of the trial, coupled with a comparison to the Keytruda+chemo control arm. Recall this is also consistent with feedback from our Therapeutics Doctor Days earlier this year (see [HERE](#)). See HARMONI-6 takeaways from our China healthcare team [HERE](#).

In our view the incremental Ph2 data at ASCO suggest that BMJ (UW)/BNTX (OW) Pumi and PFE's (EW) PF4404 are competitive with SMMT/Akeso's Ivo in NSCLC, and they achieve high ORR's across both squamous (SQ) and non-squamous (NSQ) histologies and all PD-L1 levels. See [Exhibit 6](#), [Exhibit 7](#), and [Exhibit 8](#) for cross-trial efficacy comparison, with the typical caveats for such comparisons. For Pumi, we project 2035 risk-adjusted WW sales (before profit split) of \$5.6bn, with POS ranging from 50-70% across different tumors. For PF4404, we project 2035 risk-adjusted WW sales of \$2.7bn (50% POS).

Exhibit 1: Overview of select 1L metastatic NSCLC trials

		Histology			
		Squamous	Both	Non-Squamous	
PD-L1 Status	All-comers	KEYNOTE-407 (MKR) ± Key+chemo vs. Chemo HARMONI-6 (SMMT/Akeso) ± Key+chemo vs. Teicombi+chemo Positive PFS data 10/19/25 OS data at ASCO '26	AVANZAR (AZN/Daiichi) ± Dato+rhifuzo+chemo vs. Key+chemo Topline data at JCO26 ROSETTA-Lung02 (BNTX/BMY) ± Pumit+chemo vs. Key+chemo PCD data at ASCO '26 CheckMate 5LA (BMY) ± Opdivo+Yervoy+chemo vs. Chemo HARMONI-3 (SMMT/Akeso) ± Key+chemo vs. Key+chemo SQ PFS & interim OS 24/26 NSQ final PFS 1/27	Symblinor-Lung-01 (PFE) ± PF-4404+chemo vs. Key+chemo PCD Feb-29 IPower158 (Roche) ± Tecentriq+Avastin+chemo vs. Avastin+chemo KEYNOTE-188 (MKR) ± Key+chemo vs. Chemo	
	PD-L1 ≥ 50% (PD-L1-High)		KEYNOTE-024 (MKR) ± Key vs. Chemo HARMONI-7 (SMMT/Akeso) ± Key vs. Key PCD of Apr. '26 TROPION-Lung08 (AZN/Daiichi) ± Dato+Key vs. Key Data expected ~2H26	Bela-Lung-02 (PFE) ± Siv+Key vs. Key PCD of Sept. '26 Trofus-007 (MKR/Kelun) ± Sac-TMT+Key vs. Key EVOKE-03 (GILD) ± Trodeloy+Key vs. Key Update expected at JCO26	
	PD-L1 1-49% (PD-L1-Intermediate)	ARTEMIDE-Lung02 (AZN) ± rhlgG2+chemo vs. Key+chemo PCD Feb. '29	KEYNOTE-042 (MKR) ± Key vs. Chemo OptisTOP-Lung05 (MKR/Kelun) ± Sac-TMT+Key vs. Key PFS stat. sig., OS trend 1.1/24/25 Full data at ASCO '26 CheckMate 227 (BMY) ± Opdivo+Yervoy vs. Chemo Part 2a of trial; PD-L1 ≥ 1% HARMONI-5 (SMMT/Akeso) ± Key vs. Key Positive PFS and interim OS 4/25/25 Awaiting update OS	IPower110 (Roche) ± Tecentriq vs. Chemo Approved in PD-L1 ≥ 50%	
	PD-L1 < 1% (PD-L1-Negative)				TROPION-Lung07 (AZN/Daiichi) ± (PD-L1-50%) Dato+Key+chemo vs. Key+chemo Data expected ~2H26 OptisTOP-Lung06 (MKR/Kelun) ± Sac-TMT+Key vs. Key+chemo Interim data at ESMO '26 or later
± Ongoing ± Reported					
Near-term focus ASCO 2026					

Source: Company data, CT.gov, Morgan Stanley Research

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Akeso is covered by Jack Lin; AstraZeneca is covered by Sarita Kapila; BeOne is covered by Sean Laaman; Daiichi Sankyo is covered by Shinichiro Muraoka; Kelun is covered by Alexis Yan; SMMT is not covered.

MRK/Kelun's Sac-TMT (TROP2 ADC)

China-only Ph3 sac-TMT+Keytruda vs. Keytruda data in 1L PD-L1 positive NSCLC (OptiTROP-Lung05)

MRK's partner Kelun presented Ph3 China-only data from the OptiTROP-Lung05 trial of sac-TMT+Keytruda vs. Keytruda in 1L PD-L1-positive NSCLC; see our initial take on the abstract [HERE](#). **At median follow-up of 10.5mos, median PFS was not reached for sac-TMT vs. 5.7mos for Keytruda ($p<0.0001$). PFS data to date imply a mPFS for sac-TMT +Keytruda of ~15-17mos, which is generally in line with investor expectations of ~15-20mos; see [Exhibit 2](#), and our prior estimate of ~16mos+ based on the HR.** The PFS performance of the Keytruda control arm also appears in line with expectations (see [Exhibit 10](#)). The data for OS were not mature, and a favorable trend was observed in the sac-TMT+Key group (HR, 0.55; 95% CI, 0.36-0.85). The BICR-assessed ORR was 70.2% in the sac-TMT + Key group versus 42.0% in the Key group. In the pre-specified PD-L1 subgroups, the HRs for PFS in pts with TPS 1-49% and TPS $\geq 50\%$ were 0.28 (95% CI, 0.19-0.41) and 0.47 (95% CI, 0.29-0.77). In the pre-specified histology subgroups, the HRs for PFS in pts with non-squamous and squamous were 0.28 (95% CI, 0.18-0.43) and 0.44 (95% CI, 0.29-0.66). The Lancet publication noted that the observed benefit of sac-TMT +Keytruda in patients with non-squamous NSCLC is consistent with findings from other studies of TROP2 ADCs, citing the global Ph3 TROPION-Lung01 trial of Datroway vs. chemo in 2L+ NSCLC.

Exhibit 2: Model predictions for PFS in sac-TMT+Key arm

Table S1. Parametric model predictions for progression-free survival in the sac-TMT plus pembrolizumab group

Parametric Model	Predicted progression-free survival in the Sac-TMT + Pembrolizumab group, median (95% CI)			Difference*, month	AIC
	Overall	PD-L1 TPS of 1 to 49%	PD-L1 TPS of 50% or greater		
Lognormal	16.7 (12.8–22.1)	15.7 (11.2–21.8)	18.4 (11.6–29.9)	11.0	552.0
LogLogistic	15.7 (12.4–20.0)	14.8 (11.1–20.2)	17.3 (11.3–27.2)	10.0	555.2
Exponential	17.5 (13.8–22.4)	16.9 (12.3–22.8)	18.5 (12.7–26.5)	11.8	560.4
Gamma	15.0 (12.0–18.3)	14.2 (10.9–18.4)	16.4 (11.2–22.7)	9.3	556.0
Weibull	15.0 (12.2–18.4)	14.1 (10.9–18.1)	16.5 (11.4–23.8)	9.3	556.9

* The difference in median progression-free survival between the sac-TMT plus pembrolizumab group and the pembrolizumab group was based on the predicted median value for the sac-TMT plus pembrolizumab group minus the observed median value for the pembrolizumab group in the intent-to-treat population. The observed median progression-free survival for the pembrolizumab group was 5.7 months (95% CI, 4.3–7.0). Smaller AIC indicating better fitting.

Sac-TMT=sacituzumab tirumotecan; AIC=Akaike Information Criterion; PD-L1=programmed death ligand 1; TPS=tumor proportion score; CI=confidence interval.

Source: Xiong et al, Lancet 2026

Per the publication, the safety profile of sac-TMT+Key was consistent with the known safety profiles of the individual agents, with no new safety signals identified. The increased toxicity with the combo was primarily driven by expected sac-TMT-related heme tox and stomatitis vs. Keytruda monotherapy. This heme tox was generally manageable with supportive care, dose modification, or temporary treatment interruption, while permanent discontinuation rates were low **(TEAEs led to discontinuation of sac-TMT/Keytruda in 3.8%/5.3% of pts in the sac-TMT+Keytruda group while discontinuation of Keytruda occurred in 4.9% of pts in the Keytruda mono group)**. Per the authors, taken together with the observed delay in deterioration of global health status, these findings suggest that the clinical benefit of the combination was not offset by unacceptable toxicity. Gr3+ TEAEs were 55.3% in the sac-TMT+Keytruda group and 31.4% in the Keytruda group. Most common Gr3+ TEAEs of special interest for sac-TMT were neutrophil count decreased (17.3%), anemia (9.1%), and stomatitis (5.3%).

MRK has previously commented that sac-TMT is differentiated from other TROP2 ADCs by its novel bifunctional linker design, which aims to balance stability with

payload release, with the goal of potentially mitigating the hematologic toxicity observed with GILD's Trodelvy's looser linker, while avoiding the interstitial lung disease (ILD) risk associated with tighter linker systems such as AZN/Daiichi's Datroway. See [Exhibit 11](#).

In our view, the more definitive sac-TMT data will come from global Ph3 trials. MRK/Kelun are pursuing a broad registrational development program for sac-TMT across NSCLC, breast cancer, gynecologic cancers (incl. endometrial, cervical, and ovarian), as well as urothelial cancer and gastroesophageal adenocarcinoma; see [Exhibit 12](#) for a summary. Recall, MRK recently announced positive interim PFS/OS results for the TroFuse-005 trial in 2L+ endometrial cancer; [LINK](#). We view the breadth of the program as a differentiator for sac-TMT, as successful execution across multiple tumor types could drive a meaningfully larger TAM and peak sales opportunity relative to competitors.

For MRK we model sac-TMT 2035 risk-adjusted sales of \$4.1bn (at a 55% POS) vs. MRK VA cons' of \$5.4bn. We'd note that the FDA granted MRK a Commissioner's National Priority Voucher for sac-TMT, which could enable a faster review once filed in the US.

SMMT/Akeso's Ivonescimab (PD-1xVEGF)

China-only Ph3 Ivo+chemo vs. Tevimbra+chemo data in 1L SQ NSCLC (HARMONi-6)

Background: SMMT/Akeso presented full PFS and OS data from the HARMONi-6 trial of Ivonescimab (Ivo)+chemo vs. Tevimbra (BeOne's PD-1)+chemo in 1L SQ NSCLC. We'd note that PFS data from the trial had previously been reported at ESMO in October '25 (11.1mos for Ivo+chemo vs. 6.9mos for Tevimbra+chemo, HR=0.6 p<0.0001; [LINK](#)), and thus the focus here was on OS. However, the trial uses Tevimbra+chemo rather than Keytruda +chemo as a comparator and hence investors are likely to somewhat discount the results in our view. We'd also caveat that these data are from a China-only population and will need to be replicated on a global basis. SMMT/Akeso are running a Ph3 global study in 1L NSCLC (HARMONi-3) against Keytruda + chemo, and final PFS and interim OS are expected in 2H26. Recall the Independent Data Monitoring Committee recently conducted an interim PFS analysis and recommended the study continue as planned.

At a high level in HARMONi-6, Ivo + chemo achieved a stat sig and meaningful OS benefit, which is the first evidence that the PFS benefits previously seen with the PD(L)1xVEGF class might translate into an OS benefit. However, based on the discussant's comments, we believe there are still a number of outstanding questions, including the short follow-up and whether these results can be reproduced in a global study (HARMONi-3 study) against Keytruda + chemo.

Key takeaways from HARMONi-6 presentation. At data cutoff (Feb 27, 2026) for the interim overall survival analysis, the median duration of follow-up was 21.4 months. See [Exhibit 7](#) for 1L SQ NSCLC cross-trial efficacy comparison, and [Exhibit 9](#) for 1L SQ NSCLC cross-trial safety comparison, with the typical caveats of such comparisons.

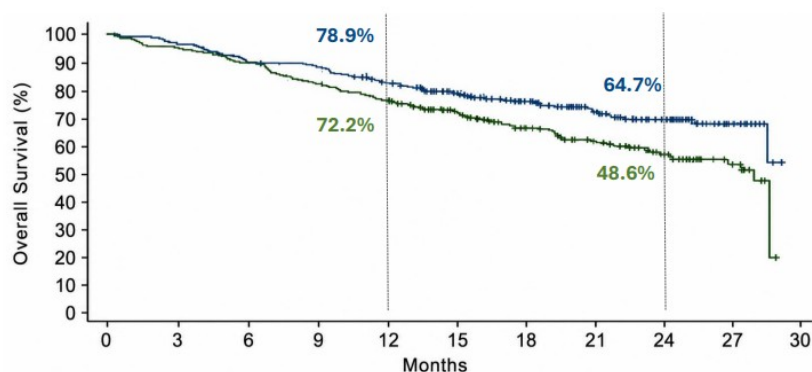
- In the ITT population, Ivo + chemo resulted in an mOS of 27.9 months vs. 23.7 months for Tevi + chemo, with a HR of 0.66 ([Exhibit 3](#)). For reference, the historical mOS for Tevi + chemo in this indication was 22.8 months in Ph3 RATIONALE-307 ([LINK](#)). We note as for Keytruda + chemo, the mOS was 15.9 months (KN-407) and 30.1 months (KN-407 China).
- The presenter noted that the mOS in the Ivo arm would have not been reached without the last single event.
- The overall survival benefit was consistent across all key subgroups
 - PD-L1 TPS <1%, the HR was 0.64
 - TPS ≥1%, the HR was 0.68
 - TPS 1-49%, the HR was 0.67
 - TPS ≥50%, the HR was 0.64
- Earlier this year we spoke with a couple lung KOLs, and they had varying degrees of excitement about the PD(L)1xVEGF bispecific drug class. Specifically on HARMONi-3 (global), they want to see an HR for OS of 0.7 to 0.75, which in their view would be transformational ([LINK](#)).
- Ivo + chemo showed a manageable safety profile, with 69% Gr≥3 TRAEs vs. 59% for the control arm. Discontinuation rate was 4% vs. 4% for the control arm ([Exhibit 5](#)). VEGF-related AEs occurred more frequently in the Ivo arm, most of which were Gr1-2. See hemorrhage rate in [Exhibit 9](#).

The independent discussant stated that while the results were provocative, there are some outstanding questions. Specifically, she provided 3 high-level takeaways: (1) Ivo + chemo is effective in Chinese patients, based on the interim OS data. (2) Ivo resulted in VEGF-related AEs, and there is uncertainty regarding patient selection. (3) Applicability to global, generally older, SQ lung cancer population is unclear. Below we highlight the key questions she raised:

- The median follow-up was only 21 months, and it is possible that with longer follow-up, the mOS and HR results may change.
- Noted the control arm mOS differences vs. Keytruda + chemo. See above.
- Unclear if the HARMONi-6 population is representative of the global patient population
 - Higher percentage of men in HARMONi-6 (96%) vs. 79% for KN-407. The HARMONi-6 presenter noted that it is due to men being the exclusive heavy smokers in China.
 - HARMONi-6 has a smaller % of PDL1 high patients compared to other 1L SQ NSCLC studies.
 - Notably, patients age 65 or older did not experience a survival benefit, where the OS HR was 0.93 for ≥ 65 vs. 0.43 for < 65 yo ([Exhibit 4](#)). For reference, we note the OS HR dispersion for these age groups was smaller in KN-407 - 0.74 for ≥ 65 vs. 0.52 for < 65 yo. She mentioned that the median age of the study was 64 (patients over 75 were excluded) vs. ~ 70 in the US, so the results limit applicability to the global population in her view.

The results were also published in [Lancet \(HERE\)](#), which echoed some of the discussant's comments: "The median overall survival was reached following the last single event, and the long-term survival tail should be interpreted with caution. Longer follow-up is therefore warranted to fully characterise the tail of the survival curve and further validate the durability of the long-term survival outcome. Second, the study enrolled relatively few female patients and excluded individuals aged over 75 years. Accordingly, the efficacy results should be interpreted with caution when extrapolated to these populations. In addition, the study population was restricted to patients from China, but a global phase 3 trial (HARMONi-3) is currently ongoing."

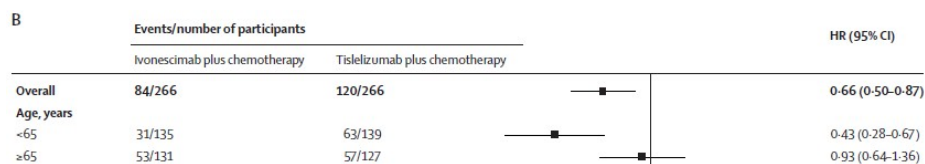
Exhibit 3: OS curves from the ITT population of HARMONi-6



	Ivonescimab + chemo (N=266)	Tislelizumab + chemo (N=266)
mOS, months (95% CI)	27.89 (27.89, NE)	23.69 (20.11, NE)
Stratified HR (95% CI)	0.66 (0.50, 0.87)	
p-value	0.0017	

OS significance boundary: 0.0049
The median OS in the ivonescimab group would have not been reached without the last single event

Source: Company Data

Exhibit 4: HARMONi-6 OS subgroup analysis by age

Source: Lu et al. (Lancet 2026)

Exhibit 5: Safety profile of Ivo + chemo in HARMONi-6

	Ivonescimab + chemo (N=266)	Tislelizumab + chemo (N=265)
TRAE	264 (99.2)	263 (99.2)
Grade ≥ 3 TRAE	184 (69.2)	156 (58.9)
Serious TRAE	110 (41.4)	91 (34.3)
Leading to ivonescimab or tislelizumab discontinuation	14 (5.3)	12 (4.5)
Leading to death	10 (3.8)	11 (4.2)
Grade ≥ 3 irAE [#]	34 (14)	36 (14)

• Data are n (%)

• # immune-related adverse events were assessed by investigators

Source: Company Data

BMY/BNTX and PFE PD-(L)1 x VEGF bispecs

BMY/BNTX presented the first global data from the PD(L)1xVEGF class in 1L NSCLC from a Ph2 dosing-finding study of Pumi + chemo in 1L NSCLC irrespective of PD-L1 levels (Rosetta-Lung02). PFE also presented updated Ph2 dosing-finding data of PF'4404 (PD1xVEGF) in 1L PD-L1 $\geq$ 1% NSCLC (China only).

At a high level, we believe these data suggest that Pumi and PF'4404 are competitive with SMMT/Akeso's Ivo, and they achieve high ORRs across both squamous (SQ) and non-squamous (NSQ) histologies and all PD-L1 levels. See [Exhibit 6](#), [Exhibit 7](#), and [Exhibit 8](#) for cross-trial efficacy comparison, with the typical caveats for such comparisons.

Takeaways from Pumi's global Ph2 dose-finding study in 1L NSCLC irrespective of PD-L1 levels (Rosetta-Lung02). The study evaluated Pumi 1400mg and 2000mg + chemo in 44 patients, in which 48% SQ and 52% NSQ. Across these patients, PD-L1 TPS<1% was 58%, TPS 1-49% was 25%, and TPS $\geq$ 50% was 17%. The safety profile appears manageable.

- ORR was higher for the 1400mg dose than the 2000mg dose across histologies. Based on the overall trial data, 1500mg Q3W flat dose is chosen for Ph3 portion of the trial, slightly above the 1400mg dose in the Ph2 study.
- For the 1400mg dose, cORR of 73% for SQ and 64% for NSQ. 100% disease control.
- Combined doses: cORR: 48% for TPS < 1%; 78% for TPS 1 – 49 %; 100% for TPS $\geq$ 50%.
- Not surprising in our view, there were more Pumi-related TRAE for SQ (91%) vs. 64% for NSQ; higher discontinuation rate due to TRAE for SQ (24%) vs. 14% for NSQ; and higher bleeding for SQ (33%) vs. 9% for NSQ. For the SQ cohort, there was one patient death, who had multiple comorbidities and was oxygen dependent, cause of death unknown. See [Exhibit 9](#) for 1L SQ NSCLC cross-trial safety comparison (with the typical caveats for such comparisons), and we note Pumi + chemo had numerically much higher discontinuation rate (24%) vs. 3% for Ivo + chemo.

For Pumi, we project 2035 risk-adjusted WW sales (before profit split) of \$5.6bn, with POS ranging from 50-70% across different tumors.

Key takeaways from PF'4404's China-only updated Ph2 data in 1L PD-L1 $\geq$ 1% NSCLC.

The study evaluated four different doses: 5, 10, 20, and 30 mg/kg in 83 patients, in which 45% SQ and 55% NSQ. Across these patients, TPS 1-49% was 62% and TPS $\geq$ 50% was 28%. The safety profile appears manageable. The efficacy and safety data shown below are for the 10mg dose (the presentation was primarily focused on this dose level).

- 10mg/kg (n=34) was chosen as the pivotal dose.
- 68% cORR and 12.4 months mPFS.
- High cORR and durable responses were observed irrespective of histology and TPS.
 - SQ: 64% cORR; 12.4 months mPFS

- NSQ: 75% cORR; 8.9 months mPFS
- TPS 1-49%: 62% cORR; 9.6 months mPFS
- TPS $\geq$ 50%: 77% cORR; 15.8 months mPFS
- Majority of the AEs were mild to moderate, with 15% Gr3+ VEGF-related AEs, and 6% Gr3+ immune-related AEs.
- 3% discontinuation rate due to TRAE.

For PF4404, we project 2035 risk-adjusted WW sales of \$2.7bn (50% POS).

Exhibit 6: 1L PD-L1 $\geq$ 1% NSCLC cross-trial efficacy comparison

1L PD-L1 $\geq$ 1% NSCLC	n	ORR	DOR	PFS	PFS HR	OS	OS HR	Phase	Study	Source	Notes
Pumi	17	47%	NR	13.6		13.9		Ph1b/2a	NCT05918445	ASCO 2024	Stage IIIB-IV
Ivonescimab	198	50%	NR	11.1	0.51			Ph3	HARMONI-2	WCLC 2024	Stage IIIB-IV; China only
vs Keytruda	200	39%	NR	5.8							
PF4404/SSGJ-707	34	68%	18	12.4		NR		Ph2	NCT06361927	ASCO 2026	China only (10mg/kg dose, selected for pivotal)
MK-2010 (20mg)	11	55%							NCT06650566	AACR 2026	China only. For 30mg, 44% ORR
Sac-TMT + Keytruda	208	70%	NR	NR (at least 16)	0.35	NR		Ph3	OptiTROP-Lung05	ASCO 2026	China only
vs Keytruda	205	42%	NR	5.7							
Keytruda	637	27%		5.4		16.7		Ph3	KN-042	Keytruda label	Stage III
vs chemo	637	27%		6.5		12.1					
Tecentriq	277	31%	26.3	5.8		18.9		Ph3	IMpower110	WCLC 2020 (Herbst et al)	Stage IV
vs chemo	277	32%	5.7	5.6		14.7					

Source: Company Data, Morgan Stanley Research

Exhibit 7: 1L SQ NSCLC cross-trial efficacy comparison

1L Squamous NSCLC	n	ORR	DOR	PFS	PFS HR	OS	OS HR	Phase	Study	Source	Notes
Ivonescimab + chemo	266	76%	11.2	11.1	0.6	27.9	0.66	Ph3	HARMONI-6	ESMO 2025 ASCO 2026	China only
vs Tevimbra + chemo	266	67%	8.38	6.9		23.7					
Pumi + chemo	11	73%						Ph2	ROSETTA Lung-02	ASCO 2026	1400mg dose (1500mg selected for Ph3)
PF4404/SSGJ-707 (10mg) + c	24/13	75%/69%						Ph2	NCT06412471	SITC2025	China only
Keytruda + chemo	278	58%	7.2	6.4		15.9		Ph3	KN-407	Keytruda label	
vs chemo	281	35%	4.9	4.8		11.3					
Keytruda + chemo	65	80%	7.1	8.3		30.1		Ph3	KN-407 China	Cheng 2021 (ITO Clin Res Rep)	
vs chemo	60	43%	3.5	4.2		12.7					

Source: Company Data, Morgan Stanley Research

Exhibit 8: 1L NSQ NSCLC cross-trial efficacy comparison

1L Non-Squamous NSCLC	n	ORR	DOR	PFS	OS	Phase	Study	Source	Notes
Ivonescimab + chemo	72	54%	15.4	13.3	NR	Ph2	NCT04736823	ELCC 2024	
Pumi + chemo	11	64%				Ph2	ROSETTA Lung-02	ASCO 2026	1400mg dose (1500mg selected for Ph3)
PF4404/SSGJ-707 (10mg) + c	29	59%				Ph2	NCT06412471	SITC2025	China only
Keytruda + chemo	410	48%	11.2	8.8	NR				
vs chemo	206	19%	7.8	4.9	11.3	Ph3	KN-189	KN-189	
Tecentriq + chemo	453	46%	10.8	7.2	18.6				
vs chemo	228	32%	7.8	6.5	13.9	Ph3	IMpower130	IMpower130	Stage IV
Avastin + chemo		35%		6.2	12.3				
vs chemo		15%		4.5	10.3	Ph3	E4599	Avastin label	Stage IIIB-IV
Tecentriq + Avastin + chemo	359	55%	10.8	8.5	19.2				
vs Avastin + chemo	337	42%	6.5	7.0	14.7	Ph3	IMpower150	IMpower150	

Source: Company Data, Morgan Stanley Research

Exhibit 9: 1L SQ NSCLC cross-trial safety comparison

1L SQ NSCLC	Pumi + chemo	Ivo + chemo	Tevimbra+chemo
n	21	266	265
Any TRAE	100%	99%	99%
Gr $\geq$ 3 TRAEs	62%	69%	59%
Gr5 TRAEs	5%	4%	4%
Discontinuation due to TRAE	24%	5%	5%
PD(L)1xVEGF drug related	14%	-	-
Immune-related AEs	38%	-	-
Gr $\geq$ 3	5%	14%	14%
VEGF-related AEs	67%	60%	26%
Gr $\geq$ 3	5%	12%	3%
Haemorrhage	33%	25%	12%
Gr $\geq$ 3	5%	3%	1%
Source	Rosetta-Lung02 (ASCO 2026)	HARMONI-6 (ASCO 2026)	HARMONI-6 (ASCO 2026)

Source: Company Data, Morgan Stanley Research

Additional Exhibits

Exhibit 10: Keytruda KEYNOTE 1L NSCLC data summary across PD-L1 expression levels

Trial	KEYNOTE-189 (5-yr follow-up) 1L NSQ NSCLC		KEYNOTE-407 (5-yr follow-up) 1L SQ NSCLC		KEYNOTE-042 (5-yr follow-up) 1L NSCLC		KEYNOTE-024 (5-yr follow-up) 1L NSCLC	
Population	Keytruda+chemo	Chemo	Keytruda+chemo	Chemo	Keytruda	Chemo	Keytruda	Chemo
PD-L1 ≥50%								
n	132	70	73	73	299	300	154	151
mPFS (mos)	11.3	4.8	8.3	4.2	6.5	6.5	7.7	5.5
12-mo PFS rate	49%	17%	37%	15%	~35%*	~30%*	~40%	~15%
mOS (mos)	27.7	10.1	19.9	11.5	20.0	12.2	26.3	13.4
PD-L1 1-49%								
n	128	58	103	104	338	337		
mPFS (mos)	9.4	4.9	8.2	6.0				
12-mo PFS rate	44%	21%	40%	19%				
mOS (mos)	21.8	12.1	18.0	13.1	13.4*	12.1*		
PD-L1 ≥1%								
n					637	637		
mPFS (mos)					5.6	6.8		
12-mo PFS rate					~30%*	~30%*		
mOS (mos)					16.4	12.1		
PD-L1 <1%								
n	127	63	95	99				
mPFS (mos)	6.2	5.1	6.3	5.9				
12-mo PFS rate	27%	16%	32%	21%				
mOS (mos)	17.2	10.2	15.0	11.0				
PD-L1 all-comers								
n	410	206	278	281				
mPFS (mos)	9.0	4.9	8.0	5.1				
12-mo PFS rate	40%	18%	36%	19%				
mOS (mos)	22.0	10.6	17.2	11.6				

Most similar to Lung-06 control

Most similar to Lung-05 control

Source: Morgan Stanley Research, Garassino et al, JCO 2023 (KN-189), Novello et al, JCO 2023 (KN-407), Reck et al, JCO 2021 (KN-024), Castro et al, JCO 2022 (KN-042); *From earlier cut (Mok et al, ELCC 2019)

Exhibit 11: TROP2 ADC safety cross-trial comparison

Company	GILD	AZN/Daiichi	AZN/Daiichi
Drug	Trodelvy	Datroway	Datroway
Dose	10 mg/kg days 1&8 (21-day cycle)	6 mg/kg Q3W (21-day cycle)	6 mg/kg Q3W (21-day cycle)
Trial	Pooled	Pooled	Pooled
Population	BC/UC	BC	NSCLC
n	1,063	443	484
Discontinuation Due to AEs			
Permanent DC due to AEs (%)	2-6%*	3%	8%
Diarrhea			
Diarrhea - Any Grade (%)	64%	11%	12%
Diarrhea - Grade 3-4 (%)	11%	<1%	0%
Cytopenias			
Neutropenia - Any Grade (%)	64%		
Neutropenia - Grade 3-4 (%)	49%		
Febrile Neutropenia (%)	6%		
Decreased hemoglobin - Any Grade (%)	69%	35%	34%
Decreased hemoglobin - Grade 3-4 (%)	6%-9%*	3%	5%
Interstitial Lung Disease			
ILD - Any Grade (%)		4%	7%
ILD - Grade 3-4 (%)		<1%	1%
ILD - Fatal (%)		<1%	2%
DC due to ILD (%)		2%	4%

Trodelvy includes IMMU-132-01, ASCENT, TROPICS-02, TROPY

Datroway BC includes TROPION-Breast01, TROPION-PanTumor01 for ILD; TROPION-Breast01 only for DCs/diarrhea/decreased hemoglobin

Datroway NSCLC includes TROPION-Lung01, TROPION-Lung05, and TROPION-PanTumor01

*DC rates: IMMU-132-01: 2%; ASCENT: 5%; TROPICS-02: 6%

Gr3/4 hemoglobin decrease: IMMU-132-01: 6%; TROPICS-02: 8%; ASCENT: 9%

Source: Trodelvy and Datroway FDA labels

Exhibit 12: Sac-TMT global Ph3 program summary

	Trial	Comparator(s)	Population	Primary Endpoint	Primary completion
NSCLC	TroFuse-004	Sac-TMT vs. chemo	3L EGFRm NSCLC	PFS, OS	May '27
	TroFuse-007	Sac-TMT+Key vs. Key	1L PD-L1 TPS ≥50% NSCLC	OS	January '28
	TroFuse-009	Sac-TMT vs. chemo	2L non-squamous EGFRm NSCLC	PFS, OS	September '28
	TroFuse-019	Sac-TMT+Key+chemo vs. Key+chemo	Adjuvant NSCLC (no pCR post surgery)	DFS	February '34
	TroFuse-023	Sac-TMT+Key+chemo vs. Key+chemo	mNSCLC maintenance (post Keytruda+chemo)	OS	January '29
Breast	TroFuse-010	Sac-TMT+/-Key vs. TPC chemo	HR+/HER2- unresectable LA or mBC	PFS	July '27
	TroFuse-011	Sac-TMT+/-Key vs. TPC chemo	LA/mTNBC PD-L1 at CPS <10	PFS, OS	May '30
	TroFuse-012	Sac-TMT+Key vs. TPC Key+/-chemo	TNBC did not achieve pCR at surgery	iDFS	December '30
	TroFuse-032	Sac-TMT+Key+chemo+TPC vs. Key+chemo+TPC	Neoadjuvant TNBC or HR-low/HER2- BC	pCR, EFS	March '33
Gynecologic Cancers	TroFuse-005	Sac-TMT vs. chemo	Post platinum & I/O endometrial cancer	PFS, OS	January '28 <i>positive topline reported</i>
	TroFuse-033	Sac-TMT+Key+chemo vs. Key+chemo	pMMR endometrial cancer	PFS, OS	May '32
	TroFuse-020	Sac-TMT vs. TPC Tivdak or chemo	2L metastatic cervical cancer	ORR, OS	June '28
	TroFuse-036	Sac-TMT+Key+chemo+/-Avastin vs. Key+chemo+/-Avastin	1L maintenance cervical cancer	PFS/OS	October '31
	TroFuse-021	Sac-TMT+Avastin vs. Avastin	Non-HRD positive advanced ovarian cancer	PFS	February '33
	TroFuse-022	Sac-TMT+Avastin vs. Avastin	Platinum sensitive recurrent ovarian cancer	PFS	April '29
Other	TroFuse-031	Sac-TMT vs. chemo	LA/m 2-4L UC post Keytruda/Padcev/chemo	OS	July '29
	TroFuse-015	Sac-TMT vs. TPC chemo	3L+ adv./met. gastroesophageal adenocarcinoma	OS	January '27

Source: CT.gov, Morgan Stanley Research

Valuation Methodology and Risks

Bristol Myers Squibb Co (BMY.N)

Our 12-month price target of \$40 is based on a ~7x P/E multiple applied to our 2Q27-1Q28 EPS estimate of \$5.66. This multiple is below BMY's 10-year average (14x) and the industry (15x) but supported by our outlook for growth in 2026+.

Risks to Upside

- Sales from recently launched products are higher than projected
- Pipeline assets deliver positive data
- Competitor drugs are unsuccessful clinically/commercially
- M&A/business development fill the revenue gap

Risks to Downside

- Sales from recently launched products are lower than projected
- Pipeline assets experience setbacks
- Competitor drugs are successful clinically and/or commercially

BioNTech SE (BNTX.O)

We derive our PT from a DCF based on our forecasts of BioNTech's existing and future products through 2040E. We use a 10% discount rate and a 3.0% terminal growth rate.

Risks to Upside

- Covid-19 vaccines become mainstays and a significant portion of population gets annual booster
- Positive readouts from mRNA cancer vaccines
- BNT327 trials read out positively and take significant I/O market share

Risks to Downside

- Minimal Covid vaccine tail
- Failure of mRNA cancer vaccines readouts
- Failure of gotistobart (anti-CTLA-4 antibody)
- Failure of BNT327

Merck & Co., Inc. (MRK.N)

Our 12-month price target of \$112 is based on a 11x P/E multiple applied to our 2Q27-1Q28 EPS estimate of \$10.16. This multiple is below MRK's 10-year average (~13.7x) and the industry (15x) but supported by our outlook for growth.

Risks to Upside

- MRK develops Keytruda co-formulations and extends the tail of the franchise
- MRK's non-oncology pipeline is successful and contributes revenue towards the end of the decade
- ROI positive M&A

Risks to Downside

- MRK's Keytuda life cycle management strategy is unsuccessful
- MRK's non-oncology pipeline is unsuccessful and will not contribute revenue in the out-er-years
- Lack of M&A or ROI negative M&A

Pfizer Inc (PFE.N)

Our 12-month price target of \$28 uses a 10x P/E multiple applied to our 2Q27-1Q28 EPS of \$2.85. This is below the peer group average of ~14-15x, but in our view is appropriate given PFE's top-line growth is declining in 2026-2030 (-5% CAGR), as well as above average patent cliff exposure vs. the peer group.

Risks to Upside

- COVID-19 testing, vaccines and treatments become mainstays
- PFE/BNTX successfully develop a COVID-19+flu combination vaccine
- PFE's LOE exposed products decline slower than expected

Risks to Downside

- COVID-19 pandemic is behind us
- PFE/BNTX's combo COVID+flu vaccine does not succeed in clinical trials
- PFE's existing franchises face commercial challenges and/or pipeline setbacks

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(as of May 31, 2026)

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Stock Rating Category	Coverage Universe		Investment Banking Clients (IBC)			Other Material Investment Services Clients (MISC)	
	Count	% of Total	Count	% of Total IBC	% of Rating Category	Count	% of Total Other MISC
Overweight/Buy	1542	42%	465	51%	30%	707	43%
Equal-weight/Hold	1571	43%	369	40%	23%	723	44%
Not-Rated/Hold	3	0%	0	0%	0%	1	0%
Underweight/Sell	551	15%	86	9%	16%	201	12%
Total	3,667		920			1632	

Data include common stock and ADRs currently assigned ratings. Investment Banking Clients are companies from whom Morgan Stanley received investment banking compensation in the last 12 months. Due to rounding off of decimals, the percentages provided in the "% of total" column may not add up to exactly 100 percent.

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Stock Price, Price Target and Rating History (See Rating Definitions)

BioNTech SE (BNTX.O) - As of 06/01/26 GMT in USD
Industry : Biotechnology



Stock Rating History: 12/16/21 : E/I; 11/5/22 : E/A; 9/23/24 : O/A

Price Target History: 12/16/21 : 294; 1/18/22 : 284; 1/31/22 : 217; 4/12/22 : 197; 5/9/22 : 195; 7/14/22 : 194; 11/7/22 : 203; 1/23/23 : 216; 3/27/23 : 150; 5/9/23 : 124; 7/11/23 : 119; 8/7/23 : 116; 10/17/23 : 110; 11/7/23 : 111; 12/12/23 : 112; 2/2/24 : 109; 5/6/24 : 101; 8/5/24 : 93; 9/23/24 : 145; 3/10/25 : 139; 4/8/25 : 140; 5/5/25 : 132; 7/10/25 : 133; 10/10/25 : 131; 11/3/25 : 134; 3/10/26 : 125; 4/10/26 : 126

Source: Morgan Stanley Research Date Format : MM/DD/YY Price Target — No Price Target Assigned (NA)

Stock Price (Not Covered by Current Analyst) — Stock Price (Covered by Current Analyst) —

Stock and Industry Ratings (abbreviations below) appear as ♦ Stock Rating/Industry View

Stock Ratings: Overweight (O) Equal-weight (E) Underweight (U) Not-Rated (NR) No Rating Available (NA)

Industry View: Attractive (A) In-line (I) Cautious (C) No Rating (NR)

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Bristol Myers Squibb Co (BMJ.N) - As of 06/01/26 GMT in USD
Industry : Major Pharmaceuticals



Stock Rating History: 6/1/21 : E/A; 4/6/22 : U/I

Price Target History: 4/30/21 : 62; 7/28/21 : 71; 10/12/21 : 66; 4/6/22 : 64; 5/2/22 : 63; 7/27/22 : 61; 10/26/22 : 60; 2/2/23 : 62; 4/27/23 : 59; 7/27/23 : 57; 10/10/23 : 56; 10/26/23 : 50; 12/12/23 : 42; 2/4/24 : 41; 4/9/24 : 40; 4/25/24 : 37; 10/31/24 : 36; 11/11/24 : 39; 2/6/25 : 37; 4/8/25 : 36; 7/10/25 : 34; 10/30/25 : 36; 12/12/25 : 37; 2/5/26 : 40

Source: Morgan Stanley Research Date Format : MM/DD/YY Price Target — No Price Target Assigned (NA)

Stock Price (Not Covered by Current Analyst) — Stock Price (Covered by Current Analyst) —

Stock and Industry Ratings (abbreviations below) appear as ♦ Stock Rating/Industry View

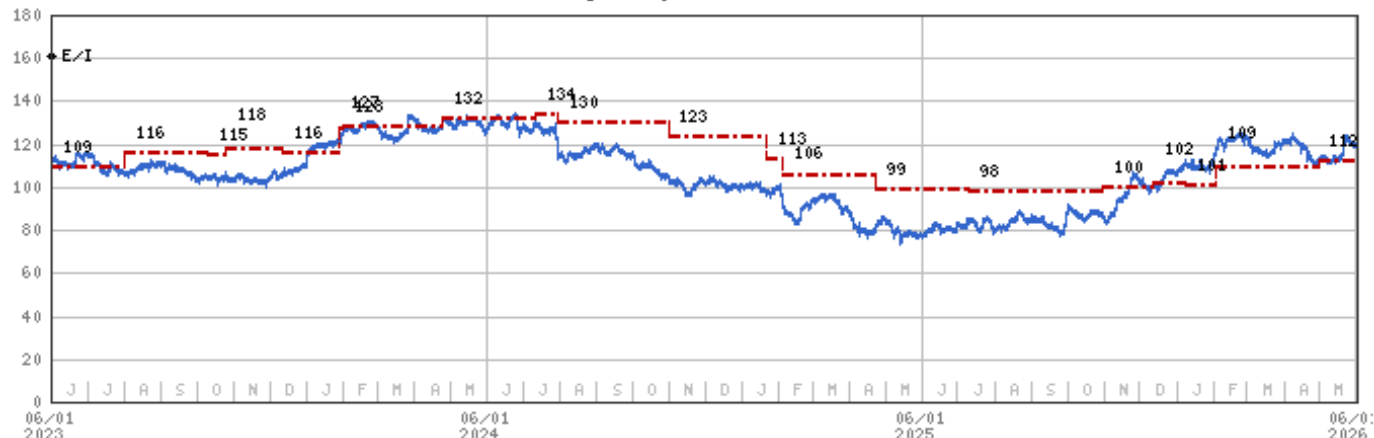
Stock Ratings: Overweight (O) Equal-weight (E) Underweight (U) Not-Rated (NR) No Rating Available (NA)

Industry View: Attractive (A) In-line (I) Cautious (C) No Rating (NR)

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Merck & Co., Inc. (MRK.N) - As of 06/01/26 GMT in USD
Industry : Major Pharmaceuticals



Stock Rating History: 6/1/21 : O/A; 9/7/21 : E/A; 4/6/22 : E/I

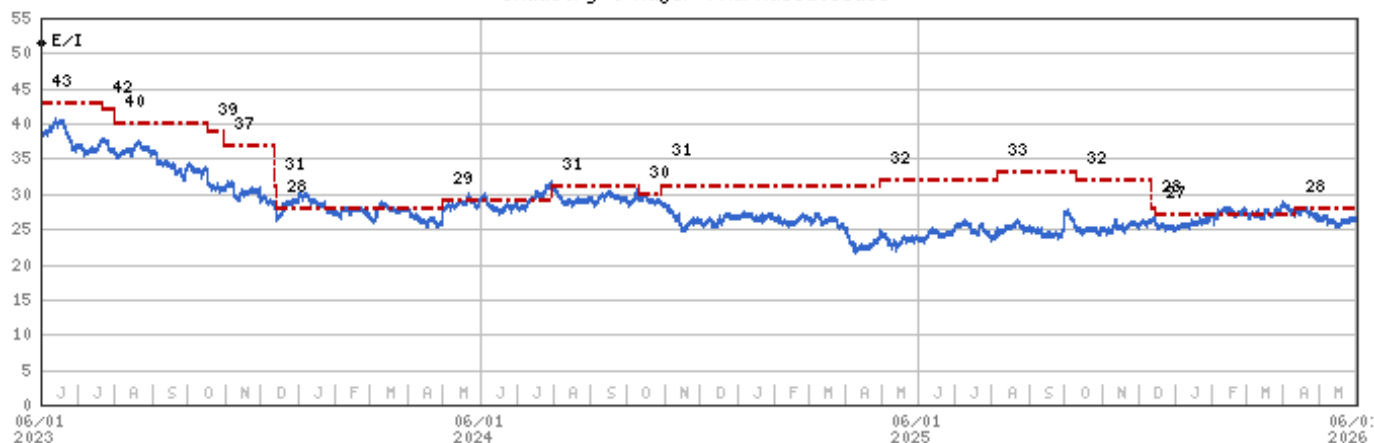
Price Target History: 12/21/20 : 90; 9/7/21 : 85; 10/12/21 : 88; 10/29/21 : 90; 11/8/21 : 88; 11/29/21 : 82; 4/6/22 : 80; 4/28/22 : 87; 7/8/22 : 88; 7/28/22 : 92; 10/12/22 : 91; 10/27/22 : 100; 2/2/23 : 99; 4/27/23 : 109; 8/1/23 : 116; 10/10/23 : 115; 10/26/23 : 118; 12/12/23 : 116; 1/29/24 : 127; 2/1/24 : 128; 4/25/24 : 132; 7/11/24 : 134; 7/30/24 : 130; 10/31/24 : 123; 1/21/25 : 113; 2/4/25 : 106; 4/24/25 : 99; 7/10/25 : 98; 10/31/25 : 100; 12/12/25 : 102; 1/8/26 : 101; 2/3/26 : 109; 4/30/26 : 112

Source: Morgan Stanley Research Date Format : MM/DD/YY Price Target --- No Price Target Assigned (NA)
Stock Price (Not Covered by Current Analyst) — Stock Price (Covered by Current Analyst) —
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Stock Ratings: Overweight (O) Equal-weight (E) Underweight (U) Not-Rated (NR) No Rating Available (NA)
Industry View: Attractive (A) In-line (I) Cautious (C) No Rating (NR)

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Pfizer Inc (PFE.N) - As of 06/01/26 GMT in USD
Industry : Major Pharmaceuticals



Stock Rating History: 6/1/21 : E/A; 4/6/22 : E/I

Price Target History: 5/9/21 : 42; 7/28/21 : 45; 10/12/21 : 48; 11/8/21 : 50; 11/29/21 : 60; 2/9/22 : 55; 5/3/22 : 52; 7/6/22 : 49; 10/12/22 : 50; 11/1/22 : 51; 12/13/22 : 53; 2/1/23 : 45; 4/10/23 : 44; 5/2/23 : 43; 7/21/23 : 42; 8/1/23 : 40; 10/17/23 : 39; 10/31/23 : 37; 12/12/23 : 31; 12/14/23 : 28; 5/1/24 : 29; 7/30/24 : 31; 10/11/24 : 30; 10/29/24 : 31; 4/29/25 : 32; 8/5/25 : 33; 10/10/25 : 32; 12/12/25 : 28; 12/16/25 : 27; 4/10/26 : 28

Source: Morgan Stanley Research Date Format : MM/DD/YY Price Target --- No Price Target Assigned (NA)
Stock Price (Not Covered by Current Analyst) — Stock Price (Covered by Current Analyst) —
Stock and Industry Ratings (abbreviations below) appear as ♦ Stock Rating/Industry View
Stock Ratings: Overweight (O) Equal-weight (E) Underweight (U) Not-Rated (NR) No Rating Available (NA)
Industry View: Attractive (A) In-line (I) Cautious (C) No Rating (NR)

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